



University of Essex

Online

Health Psychology

Seminar 4: Illness and treatment beliefs

Date



Overview of module

Unit	Topic	Assessment
1	Introduction to Health Psychology	
2	Understanding and Predicting Health Behaviours	
3	Health Promotion Interventions	CLQ poster and summary Peer responses Deadline:
4	Individual Differences	
5	Stress, Health and Coping	
6	Chronic Illness: Help-Seeking, Diagnosis, Adjustment	
7	Illness and Treatment Beliefs	
8	Psychological Processes at the End of Life	
9	The role of Psychology in Supporting Healthcare Professionals	Essay assignment: 2200 words: Deadline:



Recap Seminar 2

Stress, health and coping

Impact of stress on health

Different models of coping

Impact of coping with chronic illness



Aims of the seminar

This week's seminar will examine how our thoughts and beliefs about illness can predict how we cope and manage the illness. We will also consider the concept of healthy ageing and explore psychological processes at the end of life.

You should prepare in advance for these discussions using information from the core textbooks and recommended reading.

This week we will **test your knowledge on:**

1. Leventhal's self-regulation model. How well does Leventhal's self-regulation model explain common reactions to health threats?
2. Psychological impact of ageing.



Question 1

Why do our thoughts and feelings about illness matter?



Thoughts about illness

How do you respond to an illness threat?

Do you worry about how you will cope?

How might this impact your behaviour?





Illness cognitions/representations

Leventhal et al. (1980; 1997) defined illness cognitions as ‘a patient’s own implicit commonsense beliefs about their illness’

Illness reps have two sets of properties:

content & organisation

Illness reps define the problem:

The disease the individual is attempting to prevent, cure or control, and thereby the direction for action and specific goals and criteria for evaluating outcomes



The content of illness representations

Leventhal et al., (1980, 1997)

Identity - the label of the threat and its symptoms experienced (e.g., cold, runny nose)

Perceived cause - stress, blow to the breast, exposure to the cold

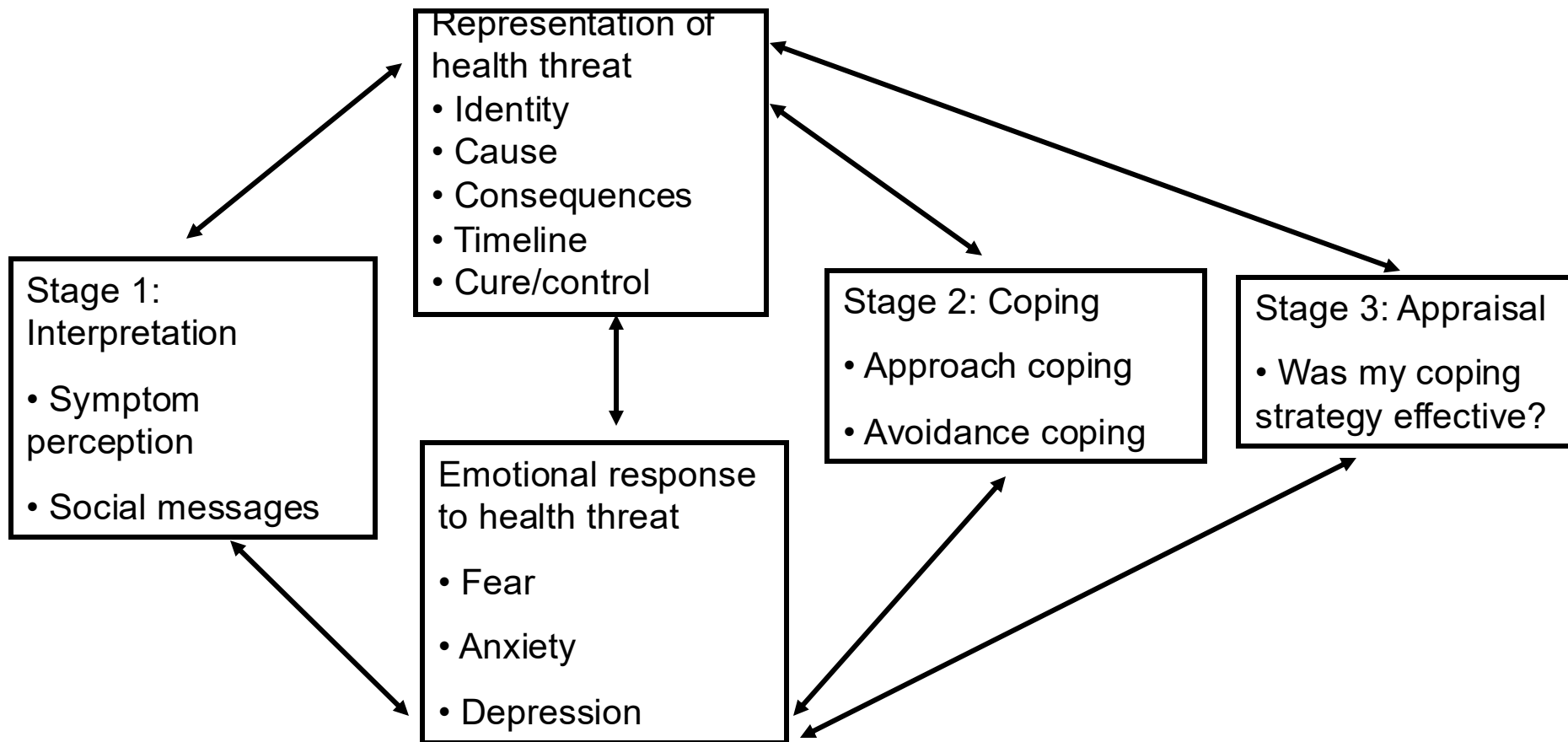
Time-line - time for development of disease, duration, time for recovery

Consequences - imagined & real (sudden death, sexual dysfunction, loss of work time)

Control/Cure - degree to which disease can be prevented, cured, kept from progressing



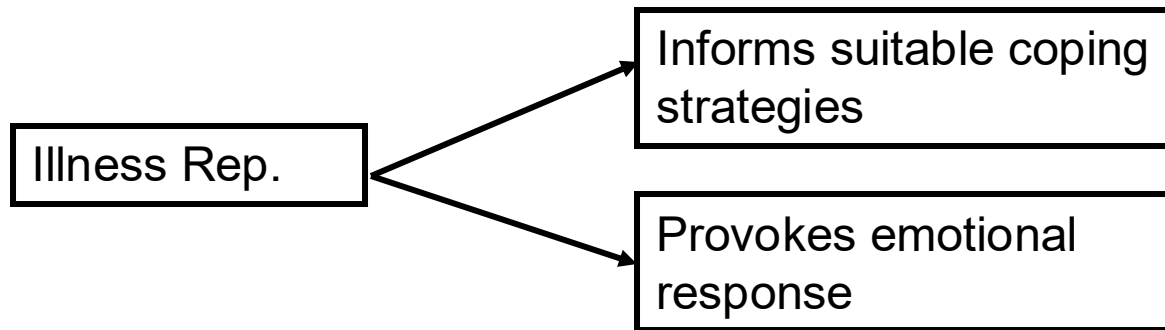
Self-regulatory model of illness





Self-regulatory model of behaviour

- **Interpretation**: Individual is confronted with the problem of potential illness
- After **interpretation** individual is motivated to return to a 'problem free' state (health).



Appraisal: evaluation of effectiveness of coping strategy - continue with the same or change?



Support for the model

Moss-Morris et al. (1996) examined relationship between illness representations, coping & psychological adjustment in chronic fatigue sufferers (CFS)

Finding: Illness representations were related to coping & outcome measure (level of functioning)

Hagger & Orbell (2003) meta-analysis: found support for inter-relationship between Illness Perceptions(IP) Questionnaire dimensions, IP-illness outcomes & IP-coping behaviour

But couldn't analyse IP-coping-illness outcome



Effects of changing illness perceptions

Changing Illness Perceptions After Myocardial Infarction: An Early Intervention Randomized Controlled Trial

KEITH J. PETRIE, PhD, LINDA D. CAMERON, PhD, CHRIS J. ELLIS, BM, DEANNA BUICK, PhD AND JOHN WEINMAN, PhD

Objective: This study was designed to examine whether a brief hospital intervention designed to alter patients' perceptions about their myocardial infarction (MI) would result in a better recovery and reduced disability. **Design:** In a prospective randomized study, 65 consecutive patients with their first MI aged were assigned to receive an intervention designed to alter their perceptions about their MI or usual care from rehabilitation nurses. Patients were assessed in hospital before and after the intervention and at 3 months after discharge from hospital. **Results:** The intervention caused significant positive changes in patients' views of their MI. Patients in the intervention group also reported they were better prepared for leaving hospital ($p < .05$) and subsequently returned to work at a significantly faster rate than the control group ($p < .05$). At the 3-month follow-up, patients in the intervention group reported a significantly lower rate of angina symptoms than control subjects (14.3 vs. 39.3, $p < .03$). There was no significant differences in rehabilitation attendance between the two groups. **Conclusions:** An in-hospital intervention designed to change patients' illness perceptions can result in improved functional outcome after MI. **Key words:** myocardial infarction, illness perceptions, return to work, disability, rehabilitation.

A picture of health—myocardial infarction patients' drawings of their hearts and subsequent disability

A longitudinal study

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Abstract

Objective: The aim of this study was to examine whether myocardial infarction (MI) patients' drawings of their hearts predict subsequent return to work, exercise, distress about symptoms and perceptions of recovery at 3 months. **Methods:** Just prior to hospital discharge, 74 consecutive patients aged under 70 admitted with an acute MI drew pictures of their hearts. Patients' recovery was assessed at 3 months by postal questionnaire. **Results:** Patients who drew damage on their heart while in the hospital perceived that their heart had recovered less at 3 months ($P=.005$), that their heart condition would last longer ($P=.01$) and had lower perceived control over their heart condition ($P=.05$) than did patients who drew no damage. The

amount of damage drawn on the heart was also associated with a slower return to work ($r=.37$, $P<.05$). While patients' peak troponin-T in the hospital was associated with the amount of damage drawn ($r=.41$, $P<.001$), it was not associated with the speed of return to work or other 3-month outcomes, apart from perceived duration of heart condition ($r=.26$, $P<.05$). **Conclusion:** Patients' drawings of damage on their hearts after a MI predict recovery better than do medical indicators of damage. Drawings offer a simple starting point for doctors to assess patients' ideas when discussing their heart condition and an opportunity to counter negative illness beliefs.

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Keywords: Myocardial infarction; Coronary heart disease; Drawings; Illness perceptions; Return to work; Recovery beliefs

Myocardial Infarction patients heart drawings

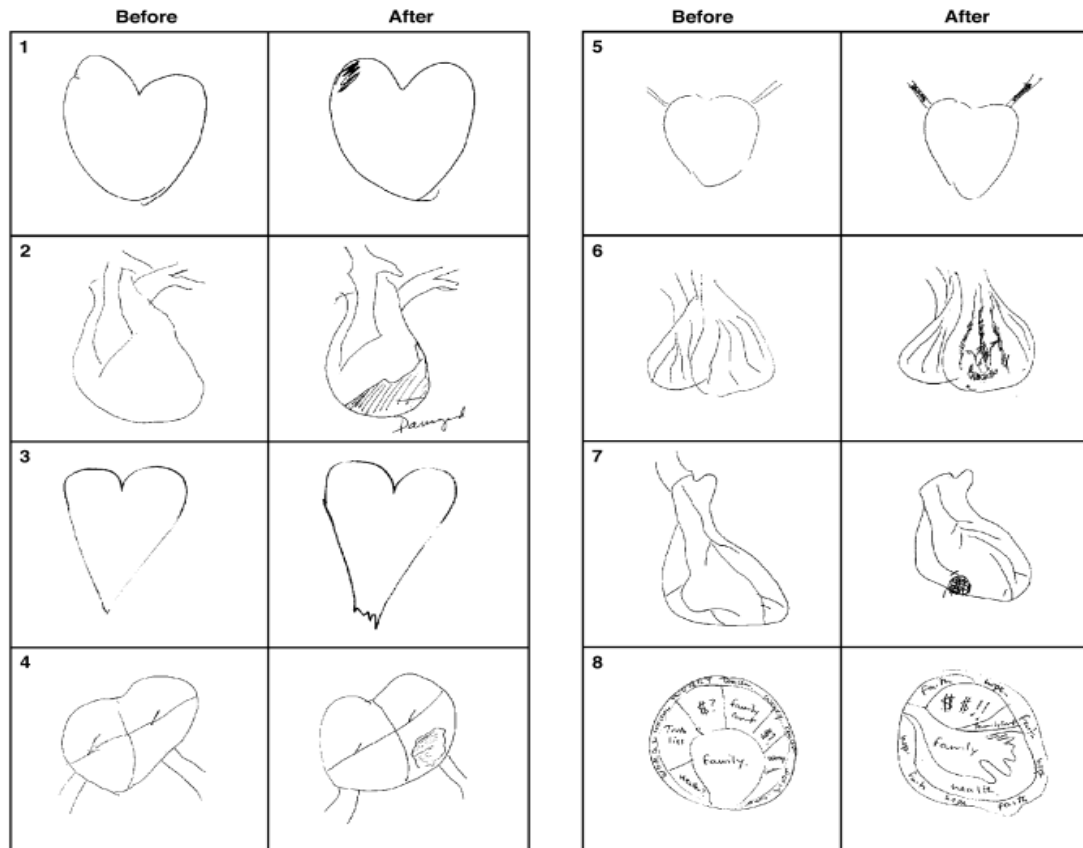


Fig. 1. Examples of heart drawings.

Broadbent et al. (2004). *Journal of Psychosomatic Research*, 57, 583-587.

Damage vs no damage

Table 1
Patients drawing damage on the heart v no damage and three month outcomes

3-Month outcome data	No damage (<i>n</i> = 29) Median (95% CI)	Damage (<i>n</i> = 24) Median (95% CI)	Significance
Days until working	21 (6 to 49)	37 (20 to 67)	.06
Perceived amount heart has recovered	9 (8 to 9)	8 (7 to 8)	.005
Perceived duration of heart condition	5 (2 to 9)	9 (8 to 10)	.01
Distress about symptoms	1 (1 to 2)	3 (1 to 5)	.10
Perceived control over heart condition	8 (7 to 9)	7 (5 to 8)	.05
Minutes spent exercising per week	47.5 (20 to 100)	120 (20 to 240)	.17

Broadbent et al. (2004). *Journal of Psychosomatic Research*, 57,
583-587.

Leventhal's self-regulatory Model applied to breast cancer

Susana Fernandes** / Teresa McIntyre**

* Universidade Lusófona do Porto, Porto, Portugal; ** College of Nursing and Texas Institute for Measurement, Evaluation and Statistics, University of Houston, Houston, USA

The aim of this study was to examine psychosocial adjustment to breast cancer, according to Leventhal's self-regulation model. We tried to understand the role of emotional adjustment and illness representations in coping. The participants were 74 Portuguese recently diagnosed breast cancer patients. Measures included the Portuguese versions of: The Hospital Anxiety and Depression Scale (HADS), to assess emotional adjustment (anxiety; depression); the Brief Illness Perception Questionnaire (B-IPQ), to evaluate illness perceptions; and the Mental Adjustment to Cancer Scale (Mini-MAC) to assess coping style. The principal hypothesis was that illness perceptions and emotional adjustment would be significant predictors of coping before surgery. The hypothesis was confirmed for most coping strategies. Emotional adjustment and illness representations were significant predictors of coping, which supports Leventhal's Self-Regulatory Model, in this sample. Emotional adjustment and illness perceptions were significant predictors of Helplessness/Hopelessness, Anxious Preoccupation, and Fighting Spirit coping strategies.

Key words: Leventhal's self-regulatory model, Breast cancer, Psychosocial adjustment, Coping, Illness representations, Emotional adjustment, Psycho-oncology.



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Any questions?



Question 2

What does positive ageing mean to you?



What is positive ageing?

“The process of developing and maintaining the functional ability that enables wellbeing in older age” (WHO, 2020).



<https://positivepsychology.com/positive-aging/>



Psychological impact of ageing

Conceptions of ageing globally

Individual differences;

- Attitude
- Personality type

Negative stereotypes

Psychosocial factors (family support, friendship groups)



Theoretical approaches of ageing

Jung considered later life to be a time to look inwards.

Erikson's eighth stage called integrity versus despair. Successful aging involves an "evaluation of one's life as having been fulfilling and satisfying" (Martin et al., 2015).

Reichard, Livson, & Petersen (1962) defined successful aging as "being well-adjusted."

Levenson et al.(2001) self-transcendence – acquisition of wisdom.



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Any questions?



End of Module essay



HPY July 2021

Participants

Grades

Module Home

Tutor Office

Deadline Details

Q & A Sessions

Module Resources

eBook

Unit 1

Unit 2

Unit 3

Unit 4

Unit 5

Behaviour Change Intervention Proposal Essay (2200 words)

Instructions

For this assignment, you are required to write an essay that will focus on one specific chronic illness (of your choosing) and to draw on psychological theory and current research to evidence your arguments.

Your proposal must address all of the following questions:

1. What is the impact of your selected chronic illness on the individual (including prevalence, adjustment, quality of life)?
2. What are the self-management challenges associated with your selected chronic illness, and can theoretical models tell us about the way individuals cope with chronic illness?
3. Critically evaluate current behaviour change interventions in your selected chronic illness.
4. Based on your understanding of theory, research and available interventions (covered in questions 1, 2, and 3), propose an intervention to promote optimal wellbeing in chronic illness.

Your proposal must demonstrate your ability to assimilate a range and depth of research knowledge and understanding of your selected illness and its impact. You should use appropriate psychological theories and research to support your proposal, and show the development of clear conclusions which are supported by evidence. Additionally, your proposal should be an effective critical evaluation and presentation of critical arguments related to the development of interventions.



Break down the question(s)

1. What is the impact of your selected chronic illness on the individual? (Consider prevalence, adjustment and quality of life).
 - *What does the literature say?*
2. What are the self-management challenges associated with your selected chronic illness, and can theoretical models tell us about the way individuals cope with chronic illness?
 - *e.g., adherence to medication regime, management of symptoms, changes in behaviour*



Break down the question(s)

3. Critically evaluate current behaviour change interventions in your selected chronic illness.

- Understand what is meant by intervention
(e.g., psycho-education, training, medication, exercise, mindfulness)
- *Explore the literature again – search:*

Interventionstudies+behaviourchange+diabetes/cancer/CHD

4. Based on your understanding of theory, research and available interventions propose an intervention to promote optimal wellbeing in chronic illness.

What do we mean by intervention?

Actions performed to bring about change, for example:

Workshops

Training

Coaching

1-1 therapy

Group work

Psycho-education



Any questions about end of
module assignment?



Check your understanding

Question 1:

Why do our thoughts and feelings about illness matter?

Self-regulatory model

Importance of appraisal

Challenge core beliefs

Impact on physical and psychological recovery



Check your understanding

Question 2:

What does positive ageing mean to you?

- Consider negative stereotypes – ageing. Why? Perception vs reality
- Attitude is everything
- Cultural perceptions differ a great deal worldwide. Western for example can be more negative vs conceptions of ageing in Japan rooted in Buddhism, Confucian and Taoist philosophies thus Ageing = wisdom
- www.positivepsychology.com



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Discuss and debate

(a) How well does the self-regulatory model explain common reactions to health threats?



Discuss and debate

(b) What are the common stereotypes relating to ageing and how might they be unhelpful?



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Next Seminar date: